

Negotiating for gender equality: women's reproductive health and wellbeing issues

UNISON Bargaining Support Group



Why are women's reproductive health and wellbeing issues a workplace issue?

As the UK's leading public service trade union for women, UNISON is determined to ensure that working women have all the support they need to be healthy and safe at work.

Around three-quarters of UNISON members and activists are women.

In the UK, 16.22 million women aged 16 and over were in employment in July to September 2024, according to the ONS [UK Labour Force Survey](#).

For some of these women, reproductive health issues will cause particular difficulties at work and may even lead to discriminatory action by employers, yet it's not something we usually talk about.

These health issues can also affect some transgender, non-binary and intersex people.

Many women are being driven from the workplace because they find that adapting their health issues around inflexible work expectations is just too difficult. Others may find that managing issues mean they miss out on promotions and training, reduce their hours, lose confidence in the workplace and see their pay levels drop, all contributing to a widening gender pay gap.

Under the UK government's [Employment Rights Bill](#) and [Next Steps to Make Work Pay](#) plans, large employers will also be required to produce action plans on how to address their gender pay gaps and on how they will support employees through the menopause.

Addressing the barriers experienced by women, some trans, non-binary and intersex workers related to their health issues now, will mean that employers will be better prepared for these legislative changes.

UNISON reps support members through capability procedures or when they ask for reasonable adjustments to enable them to work. However many managers can be ignorant of or unsympathetic about conditions that affect women.

Workplace sickness absence policies often use inflexible trigger levels that don't take account of these issues, setting off formal procedures that could ultimately lead to an unfair dismissal.

- By agreeing good workplace policies such as health and safety, sickness absence, flexible working and performance management, that take account of the impact of women's reproductive health issues, the number of cases requiring steward representation could be reduced, freeing up steward time.
- Improving conditions for workers who may be disadvantaged by practices that do not take account of these type of health issues may help in addressing the gender pay gap.
- Agreeing successful policies for a wide range of workers can be a useful recruitment and retention tool, advertising the benefits of joining UNISON for all. It can also highlight how UNISON reps have expert negotiation skills when dealing with employers.
- Organising around women's health issues and its impact on workers is a great way to increase involvement and participation of UNISON members in your branch.

Raising awareness in the workplace

The following list outlines some of the reproductive health issues that women and some trans, non-binary and intersex people can face.

Raising awareness of these issues among both employers and employees is a crucial step for ensuring that policies such as capability and sickness absence avoid discriminatory positions.

Infertility

One in six couples experience fertility challenges, which equates to around 3.5 million people in the UK.

This can be a stressful experience sometimes requiring specialist advice or treatment. But fertility treatment programmes often have long waiting lists with the process itself sometimes stretching over several years.

There are many potential causes of infertility, with fertility problems affecting the male as well as the female partner. Common causes of infertility in women include lack of regular ovulation (the monthly release of an egg), blockage of the fallopian tubes and endometriosis, but there are many other causes including the side effects of some types of medicines and drugs.

Types of fertility treatment available include: medical treatment for lack of regular ovulation; surgical procedures such as treatment for endometriosis, repair of the fallopian tubes, or removal of scarring (adhesions) within the womb or abdominal cavity; assisted conception such as intrauterine insemination (IUI) or in-vitro fertilisation (IVF).

Some types of infertility treatment can cause complications, including side effects of medication, increased risk of ectopic pregnancy, multiple pregnancy and stress.

However, the April 2023 report from [the Pregnant Then Screwed charity, published in partnership with Women In Data](#), found that being transparent about fertility at work can be costly to women's careers.

Less than half (42%) of women going through fertility treatment told their boss. Of those, one in four (24%) didn't receive any support from their employer, and 24% said they experienced unfair treatment as a result.

UNISON Scotland case study

"The public service union UNISON has recognised that employees undergoing investigations and treatment for infertility are often left with no choice but to take either sick leave or annual leave to cover the period of their absence. This is despite the worldwide recognition of infertility as a 'disease of the reproductive system', defined as such by the World Health Organisation, the National Institute for Health and Care Excellence (NICE) and the National Group on Infertility commissioned by the Scottish Government.

Fertility Network Scotland has worked with UNISON to implement a fertility policy to ensure that those requiring investigations and treatment for infertility are protected from workplace discrimination and are subject to the same rules as other employees who wish to take medical leave. From the end of July 2017 all 32 local authorities in Scotland had adopted this policy.”

[Fertility Network UK](#)

Some UK employers have also signed up to [The Fertility Workplace Pledge](#) designed to benefit millions of individuals and couples going through fertility treatment by providing:

1. Accessible information: Having an accessible workplace fertility policy to create an open culture free from stigma; to make sure employees feel comfortable in the workplace; and to prevent the best talent from leaving.
2. Awareness in the workplace: Establishing the role of Fertility Ambassador to open conversations internally and make people aware of available support.
3. Staff training: Making sure line managers understand the realities of treatment for employees including the physical, mental, and financial impact — and how they can support someone going through it.
4. Flexible working: Giving the right for employees to request flexible working, including reasonable working adjustments, so they can attend appointments.

Although there is currently no statutory right to paid or unpaid time off for fertility treatment, some enlightened employers have agreed to specific procedures for time off. In that way they can avoid any possible claim of sex discrimination.

Once the ovum is successfully implanted, pregnancy and maternity protections and benefits apply.

Further information:

Fertility Network UK provides free and impartial support, advice, information and understanding for anyone affected by fertility issues.

<https://fertilitynetworkuk.org/>

Fertility in the Workplace: helping firms support staff on their fertility journey

<https://fertilitynetworkuk.org/fertility-in-the-workplace-helping-firms-support-staff-on-their-fertility-journey/>

Pregnancy loss

It is crucial that staff members who experience pregnancy loss are dealt with sympathetically in the workplace. It should be recognised that such a loss at whatever stage in the pregnancy can severely affect individuals.

Pregnancy loss includes:

- Miscarriage: the spontaneous loss of pregnancy until 24 weeks of gestation
- Stillbirth: the loss of a baby after 24 weeks, before or during birth
- Abortion: a medical or surgical procedure to end a pregnancy
- Ectopic pregnancy: when a fertilised egg implants and grows outside of the uterus
- Molar pregnancy: a rare form of pregnancy in which a non-viable fertilised egg implants in the uterus and will fail to reach full term
- Blighted ovum (anembryonic pregnancy): when cells stop growing early on. Instead of developing into a baby, the embryo is reabsorbed, causing a miscarriage. The pregnancy sac, where the baby should have grown, sometimes continues to develop
- Neonatal loss: the loss of a baby within the first 28 days after they are born, often caused by premature births or genetic disorders.

Mothers who have a stillbirth after 24 weeks of pregnancy still legally qualify for full statutory maternity rights. This also applies if the baby lives for a short period of time and there is a neonatal death (up to 28 days after the birth whenever that may be). Their partners also qualify for maternity support (paternity) leave and shared parental leave already booked.

In addition, both the mother and their partner may be eligible for Parental Bereavement Leave and Pay.

But women who miscarry before 24 weeks of pregnancy are only entitled to compassionate leave and, if appropriate, to sick leave, even though pregnancy loss at any time can be an emotionally traumatic experience.

However women can take anything from a few days to a few weeks to recover physically from a miscarriage. The main sign of a miscarriage is vaginal bleeding, which may be followed by cramping and pain in the lower abdomen. It is believed that one in eight known pregnancies ends in miscarriage. Many miscarriages happen before someone knows they're pregnant.

The majority of miscarriages cannot be prevented and are mostly a one-off event. Most women go on to have a successful pregnancy in the future. Sometimes, miscarriages happen because the pregnancy develops outside the womb, known as an ectopic

pregnancy. This is potentially serious as there is a risk the woman could experience internal bleeding.

Ideally the employer should count any sick leave related to a pregnancy loss such as through miscarriage, ectopic pregnancy or abortion as pregnancy-related and therefore it should be noted separately from other sickness absence and not count towards any trigger points. The employer should not use it against employees, for example, for disciplinary or redundancy purposes.

After any pregnancy loss, the employer should also ideally provide special leave or compassionate leave to support both the woman and her partner.

Further information:

The Miscarriage Association

www.miscarriageassociation.org.uk/

SANDS – the leading stillbirth and neonatal death charity in the UK.

www.sands.org.uk

Ectopic Pregnancy Trust providing information, education, and support to those affected by ectopic pregnancy

www.ectopic.org.uk

Antenatal Results and Choices (ARC) providing support to those who decide to terminate a pregnancy for medical reasons

www.arc-uk.org/for-parents/ending-a-pregnancy/

UNISON's 'Model pregnancy loss policy'

www.unison.org.uk/?attachment_id=356773

Postnatal and antenatal depression (PND)

As many as eight out of 10 women get what's known as the 'baby blues' – a brief period of low mood, feeling emotional and tearful. It normally begins within around three to 10 days after giving birth and it is natural to feel this after experiencing childbirth and becoming a parent.

However, some new mothers develop postnatal depression (PND) which is a much deeper and longer-term depression. This usually develops within 6 weeks after birth. It can be gradual or sudden. And it can range from being mild to very severe.

Around 10 to 15% of new mothers experience a severe and longer-term depression that has a big impact on how they live their lives, and many do not recognise it or realise that it is a treatable illness.

The symptoms can include:

- Low mood and persistent sadness
- Lack of energy
- Difficulty bonding with baby
- Overeating and under eating for comfort
- Frightening and intrusive thoughts
- Lack of enjoyment and loss of interest in the wider world
- Trouble sleeping at night and feeling sleepy during the day
- Withdrawing from contact with other people
- Lack of concentration and difficulty making decisions

Depression in pregnancy (antenatal depression) is also common, affecting more than one in 10 women.

It is important that any pregnancy or childbirth-related sickness absence is not used as a reason for disciplinary or capability action, dismissal or redundancy.

All pregnancy-related sickness absence should be recorded separately from any other sickness absence by the employer so that the employee is not disadvantaged. Therefore, it's also very important that employees ask their GP or midwife for a letter or fit note for their employer that highlights how the sick leave is related to pregnancy or childbirth.

Employees should also be aware that Keeping in Touch (KIT) days are optional and employers must not treat anyone unfairly for refusing to come into work during their maternity leave.

Further information:

Pandas: PND Awareness and Support

<https://pandasfoundation.org.uk/>

Association for Post Natal Illness

<https://apni.org/>

Mind: postnatal depression and perinatal mental health

www.mind.org.uk/information-support/types-of-mental-health-problems/postnatal-depression-and-perinatal-mental-health/

Premenstrual Syndrome (PMS) and other menstrual problems

For the majority of women, menstruation is a natural process that doesn't present difficulties. However, there is still a social stigma around menstruation, and the topic is frequently avoided and concealed within society, making discussion difficult around problems experienced by women, some trans men, non-binary and intersex people causing many employees to struggle in silence.

As cited by the [British Standard 30416](#), a 2021 study of 6,812 adult women from Europe, USA and Brazil found that during menstruation over 80% experienced the following symptoms: mood changes/anxiety (90.6%), tiredness/fatigue (86.2%), abdominal pain (84.2%) and breast pain/tenderness (83.1%).

The menstrual cycle can be affected by a number of conditions that may cause serious discomfort or concerns for employees. Problems include amenorrhea (absent menstrual periods), menorrhagia (heavy menstrual periods), dysmenorrhea (painful menstrual periods).

Premenstrual syndrome (PMS) is a chronic condition experienced by menstruating women that is characterised by distressing physical, behavioural and psychological symptoms that regularly recur during the luteal phase of the menstrual cycle (from ovulation to the onset of a period) and that disappear or significantly diminish by the end of the period (menstruation). Common psychological and behavioural symptoms are: mood swings, depression, tiredness, fatigue or lethargy, anxiety, feeling out of control, irritability, aggression, anger, sleep disorder, food cravings. Common physical symptoms are: breast tenderness, bloating, cramping, weight gain, clumsiness, headaches.

Premenstrual dysphoric disorder (PMDD) is a very severe form of premenstrual syndrome (PMS). It can have a big impact on mental and physical health.

Further information:

The Menstrual Health project

<https://menstrualhealthproject.org.uk/>

National Association for Premenstrual Syndromes

www.pms.org.uk/

Premenstrual dysphoric disorder

www.mind.org.uk/information-support/types-of-mental-health-problems/premenstrual-dysphoric-disorder-pmdd/

Endometriosis and Adenomyosis

Endometriosis and adenomyosis are chronic and painful conditions for women and some trans men, non-binary and intersex people.

Adenomyosis is a condition where the lining of the womb (uterus) starts growing into the muscle in the wall of the womb.

Endometriosis is a different condition where tissue similar to the lining of the womb grows in other places, such as the ovaries or fallopian tubes instead of being lost during menstruation. It can also affect organs such as the bladder and bowel. The impact is more than gynaecological, particularly if other organs are affected.

Adenomyosis is thought to affect around one in 10 women in the UK. Endometriosis is common and affects 1.5 million women in the UK.

Symptoms are similar and can include:

- Pelvic pain
- Painful periods that interfere with everyday life
- Heavy menstrual bleeding
- Pain during or after sex
- Painful bowel movements
- Pain when urinating
- Fatigue, with one or more of the above symptoms.

Further information:

The Pelvic Pain Support Network

www.pelvicpain.org.uk

Endometriosis UK

www.endometriosis-uk.org/

Polycystic Ovary Syndrome (PCOS)

Polycystic ovary syndrome (PCOS) is a common and complex hormone (endocrine) condition that affects over 4 million in the UK. It affects how a woman's ovaries work. It can cause the ovaries to become enlarged and contain many fluid-filled sacs (follicles) that surround the eggs, as well as make periods irregular and increase androgen ('male' hormones) in the body, which may cause physical signs such as excess facial or body hair.

PCOS is thought to be very common, affecting at least 1 in every 10 women in the UK. Whilst more than half of these women do not have any symptoms, it can cause:

- irregular periods or no periods at all
- difficulty getting pregnant
- excessive hair growth
- weight gain
- thinning hair and hair loss from the head
- oily skin or acne.

Further information:

Verity: sharing the truth about PCOS

www.verity-pcos.org.uk/

PCOS Vitality

www.pcosvitality.com/

Fibroids

Fibroids are small benign tumours that grow on the walls of the womb, inside and out, and can range from a single growth to multiple. They are very common with around two in every three women developing at least one fibroid at some point in their life.

The exact cause of fibroids is unknown, but they have been linked to the hormone oestrogen. Fibroids usually develop during a woman's reproductive years (from around the age of 16 to 50) when oestrogen levels are at their highest.

For most women they give no cause for concern and grow unnoticed. But, for others, they can mean heavy and painful periods and lead to fatigue and anaemia. Other symptoms include: lower back pain, a frequent need to urinate, constipation, and pain or discomfort during sex. In the worst cases, it can make conceiving more complicated, cause infertility and increase the risk of miscarriage.

Black women suffer disproportionately from fibroids and are three to five times more likely to develop them than white women.

Further information:

British Fibroid Trust

www.britishfibroidtrust.org.uk/

Fibroid Forum UK

<https://fibroidforumuk.org/>

Menopause

The menopause is a natural transition stage in most women's lives, and in some trans, non-binary and intersex people. It is marked by changes in the hormones and the woman stops having periods. Women may also experience a wide range of physical and psychological symptoms as a result of the menopause.

In the years leading up to the menopause called the perimenopause, there can be significant changes for women, with irregular and heavy menstrual bleeding as well as many of the classic symptoms associated with menopause, including anxiety, low mood, hot flushes, brain fog and vaginal dryness, amongst many others.

Although most women experience this natural change between the ages of 45 and 55, for some women, it can be experienced at a much younger age, in their 30s or even younger. Women who experience an early menopause may also have to cope with the psychological distress of facing infertility at an early age.

Post-menopause is a term used when a woman's periods have stopped for 12 consecutive months. However, other menopause symptoms may not have ended so soon. Problematic symptoms may continue for years.

Everyone's experience of the menopause is individual and may differ greatly. But inevitably the symptoms will be exacerbated by negative or discriminatory attitudes in the workplace.

The [Fawcett Society found in their survey](#) that one in ten women who have been employed during the menopause have left work due to menopause symptoms. Mapped on to the UK population that would represent an estimated 333,000 women leaving their jobs due to the menopause. 14% of women had reduced their hours at work, 14% had gone part-time, and 8% had not applied for promotion.

Although symptoms may last a comparatively short time, they can frequently trigger formal monitoring procedures at work. And whilst symptoms may be severely debilitating for some, they are too often written off as 'women's problems' that are trivial or an embarrassing joke.

Development of a workplace policy on the menopause could go a long way to help ensure that women, as well as some transgender, non-binary and intersex people, are not disadvantaged and that experienced talent is not lost from the workforce. But policies are only useful if they are implemented and regularly reviewed.

Further information:

UNISON's 'The menopause is a workplace issue: guidance and model policy'

www.unison.org.uk/content/uploads/2021/02/26305_menopause_guide-1.pdf

Breast cancer

Breast cancer is the most common cancer in the UK, despite it being a rare disease in men (with only one in 870 men getting it).

Every year, according to Cancer Research UK, over 56,800 individuals are diagnosed – that is 156 people a day. A woman born after 1960 and living in the UK has an estimated one in seven lifetime risk of developing breast cancer.

But if detected early, there is a good chance of recovery.

Breast cancer is cancer that starts in breast tissue. It begins when abnormal cells in the breast grow uncontrolled and eventually form a growth (tumour). Breast cancer occurs when breast tumours spread. There is generally a long period between breast tissue changes and the development of breast cancer.

Therefore it is important for women to check their breasts regularly for any changes and always get any changes examined by their GP. The first noticeable symptom is usually a lump or area of thickened breast tissue.

As the risk of breast cancer increases with age, all women who are 50 to 70 years old are invited for breast cancer screening (mammographic screening or mammograms) every three years. If cancer is detected at an early stage, it can be treated (with surgery, radiotherapy and chemotherapy) before it spreads to nearby parts of the body.

The causes of breast cancer are not fully understood. However, studies have identified numerous factors which increase breast cancer risk (known as 'risk factors'), including those associated with age, genetics, circulating hormone levels, lifestyle, diet and environmental factors.

Further information:

Breast Cancer Now

<https://breastcancernow.org/>

Breast Cancer UK

www.breastcanceruk.org.uk/

CoppaFeel!

<https://coppafeel.org/>

Prevent Breast Cancer

<https://preventbreastcancer.org.uk/>

Gynaecological cancers

There are five gynaecological cancers: cervical cancer, vulval cancer, vaginal cancer, womb cancer and ovarian cancer. As cited by Wellbeing of Women, 21,000 women are diagnosed with a gynaecological cancer every year in the UK.

Cervical cancer

In the UK, there are over 3,100 cases of cervical cancer (or cancer of the cervix at the entrance to the womb from the vagina) each year, according to Wellbeing of Women charity. Cervical cancer usually grows slowly. In people with healthy immune systems, it typically takes 15-20 years for cervical cancer to develop.

Cervical cancer rates in the UK are highest in women and people with a cervix aged 30–34 years. But younger and older women can still get cervical cancer so it's important for them to attend screening appointments.

Symptoms of cervical cancer include:

- vaginal bleeding that's unusual for the person – including bleeding during or after sex, between periods or after the menopause, or having heavier periods than usual
- changes to the person's vaginal discharge
- pain during sex
- pain in the lower back, between the hip bones (pelvis), or in the lower tummy.

Cervical cancer is not thought to be hereditary. Nearly all cervical cancers are caused by persistent infections with a virus called high-risk human papillomavirus (HPV).

HPV is a very common virus transmitted through skin-to-skin contact in the genital area. Around four out of five sexually active adults (80%) will be infected with some type of HPV in their lives. However, for the majority of women this will not result in cervical cancer. It often has no symptoms in its early stages, so screening (commonly referred to as a smear test) is very important in order to detect it at an early stage or before it develops. It is usually possible to treat using surgery if diagnosed at an early stage, but may also require radiotherapy and chemotherapy.

Vulval and vaginal cancer

The vulva is the external part of the female genitals, around the opening of the vagina. Vulval cancer grows slowly and sometimes starts as a growth of abnormal cells.

Vulval cancer is rare. According to Cancer Research UK, there are around 1,400 new vulval cancer cases in the UK every year.

The risk of vulval cancer increases with age. It's most common in women over the age of 65. However, young women can get pre-cancerous changes and cancer.

Symptoms of vulval cancer can include:

- a persistent itch in the vulva

- pain, soreness or tenderness in the vulva
- raised and thickened patches of skin that can be red, white or dark
- a lump or wart-like growth on the vulva
- bleeding from the vulva or blood-stained vaginal discharge between periods
- an open sore in the vulva
- a burning pain when peeing
- a mole on the vulva that changes shape or colour.

Seven in 10 cases of vulval cancer are caused by certain types of human papilloma virus (HPV) spread through close skin-to-skin contact during any type of sexual activity with a partner. Other factors can also increase a person's risk of developing vulval cancer, including a weakened immune system.

Treatment for vulval cancer normally involves surgery and can cause side effects. These may include bladder and bowel problems and triggering early menopause.

Vaginal cancer is the least common gynaecological cancer. It is most common over the age of 75 (40%) but anyone with a vagina can be diagnosed with it at any age. Three out of four vaginal cancers are linked to the human papilloma virus (HPV).

Vaginal cancer usually grows very slowly and how serious it is depends on how big it is, if it has spread and the person's general health.

The main symptoms of vaginal cancer are usually:

- a lump in the vagina
- ulcers and other skin changes in or around the vagina.

Other symptoms of vaginal cancer include:

- bleeding from the vagina after the menopause
- bleeding after sex or pain during sex
- smelly or bloodstained vaginal discharge
- bleeding between periods
- an itch in the vagina that will not go away
- pain when you pee, or needing to pee a lot.

Vaginal cancer is often treatable. The main treatment for vaginal cancer is radiotherapy. Treatment may also involve surgery and chemotherapy.

Womb (uterus) cancer

Womb cancer affects the womb (sometimes called the uterus) and is most common in post-menopausal women.

Most womb cancer usually starts in the lining of the womb (endometrium), also known as endometrial cancer.

As cited by Wellbeing of Women, womb cancer is the fourth most common cancer in women in the UK. Around 9,700 women are diagnosed with womb cancer each year.

It is often not clear why someone gets womb cancer. Having a high level of oestrogen hormone, without adequate balance of progesterone hormone, can increase a person's risk.

Womb cancer is also strongly linked to living with overweight or obesity, with around one third of cases being due to excess body fat. After the menopause, body fat is responsible for most of the oestrogen production. However, there are other medical conditions, reproductive factors and certain inherited conditions that make womb cancer more likely to develop.

The main symptoms of womb cancer can include:

- bleeding or spotting from the vagina after the menopause
- heavy periods that are unusual for the person
- vaginal bleeding between periods
- a change to the vaginal discharge.

Surgery, radiotherapy, chemotherapy and immunotherapy are the main treatments.

Ovarian cancer

Ovarian cancer can affect anyone who has ovaries. It happens when cells in the ovary or fallopian tube become abnormal. These cells can grow and divide uncontrollably and eventually the abnormal cells can spread to other organs. It is still possible to get ovarian cancer even if a person has had their ovaries removed.

According to Target Ovarian Cancer, each year 7,400 women are diagnosed with ovarian cancer in the UK. It is most common in women over 50, but younger women can still get it. The earlier ovarian cancer is diagnosed, the easier it is to treat, but unfortunately, there is not yet any effective way to screen for ovarian cancer.

Symptoms of ovarian cancer include frequently (roughly 12 or more times a month) having:

- a swollen tummy or feeling bloated
- pain or tenderness in the tummy or the area between the hips (pelvis)
- no appetite or feeling full quickly after eating
- an urgent need to pee or needing to pee more often.

Other symptoms of ovarian cancer can include:

- indigestion
- constipation or diarrhoea

- back pain
- feeling tired all the time
- losing weight without trying
- bleeding from the vagina after the menopause.

The cause of ovarian cancer is not yet understood. The biggest risk factor for ovarian cancer is getting older and the risk rises steeply from the age of 45. It is most common in those who have been through the menopause with more than half of all cases in the UK in women aged 65 and over.

The main treatments are surgery and chemotherapy. Other treatments include targeted medicines and hormone treatments.

Further information:

The Eve Appeal: the gynaecological cancers charity

<https://eveappeal.org.uk/>

Go Girls - supporting women, men, trans people and non binary people diagnosed with a gynaecological cancer

www.gogirlssupport.org/

Lady Garden Foundation

www.ladygardenfoundation.com/

Grace: Gynae-oncology Research and Clinical Excellence

<https://grace-charity.org.uk/>

UK Cervical Cancer

<https://ukcervicalcancer.org.uk/>

Lichen Sclerosus & Vulval Cancer UK Awareness

www.lsvcukawareness.co.uk/

Peaches: Womb Cancer Trust

<https://peachestrust.org/>

Ovarian Cancer Action

<https://ovarian.org.uk/>

Ovacome: ovarian cancer

www.ovacome.org.uk/

Target Ovarian Cancer

<https://targetovariancancer.org.uk/>

Further information to support women members in the workplace:

UNISON's website section for women members

www.unison.org.uk/about/what-we-do/fairness-equality/women/

UNISON's 'Negotiating for working parents'

www.unison.org.uk/negotiating-for-working-parents

UNISON's 'Gender, safety and health: a guide for safety reps'

<https://shop.unison.site/product/gender-safety-and-health-guide-reprint/>

'Women's health and safety: a UNISON guide'

<https://shop.unison.site/product/womens-health-and-safety-a-unison-guide/>

UNISON's 'Time off for medical and dental appointments, eye tests and for health screening'

www.unison.org.uk/medical-screening-leave/

NHS information on women's health

www.nhs.uk/womens-health

Women's Health Concern is a charitable organisation that aims to help educate and support women with their healthcare by providing unbiased, accurate information.

www.womens-health-concern.org

Wellbeing of Women is a women's health charity saving and changing the lives of women, girls and babies.

www.wellbeingofwomen.org.uk/

Maternity Action is the UK's maternity rights charity dedicated to promoting, protecting and enhancing the rights of all pregnant women, new mothers and their families to employment, social security and health care.

<https://maternityaction.org.uk/>

BS 30416 - Menstruation, Menstrual Health and Menopause in the Workplace offers practical guidance for employers to support employees experiencing menstruation, menstrual health, or menopause in the workplace.

www.bsigroup.com/en-GB/insights-and-media/insights/brochures/bs-30416-menstruation-menstrual-health-and-menopause-in-the-workplace/

Bargaining checklist for a best practice approach

For the employer there is a clear financial benefit in adopting policies that consider the needs of their women employees, particularly in order:

- to retain experienced and valued staff
- to help staff manage their conditions and thereby reduce the likelihood of long-term sickness absence
- to help them achieve greater diversity within the workforce and contribute to gender pay gap action plans
- to help avoid potentially discriminatory treatment of women, including disabled, pregnant and older women, as well as some trans, non-binary or intersex people.

UNISON reps should consider negotiation of the following:

- ☐ The development of an environment of openness, sensitivity and transparency where everyone can talk about gender-specific conditions such as the menopause and menstrual problems, and where offensive banter and ridicule are clearly outlawed in the workplace.
- ☐ Raising awareness and a sympathetic understanding amongst all staff about women's reproductive health issues and the potential impact of symptoms in the workplace, also highlighting how some transgender, non-binary and intersex people can be affected. You might be able to take advantage of the various awareness raising months or days in the calendar as a stimulus for workplace activities:
 - In January there is a Cervical Cancer Awareness Week (20-26 January in 2025)
 - March 8 is International Women's Day
 - March is Ovarian Cancer Awareness Month
 - May 28 is International Day of Action for Women's Health
 - May is Women's Health Month
 - July is Fibroids Awareness Month
 - September marks Polycystic Ovary Syndrome (PCOS) Awareness Month
 - September is Gynaecological Cancer Awareness Month: cervical cancer, vulval cancer, vaginal cancer, womb cancer and ovarian cancer
 - 15 October is Baby Loss Awareness Day
 - In October there is a Baby Loss Awareness Week (9-15 October in 2025)
 - 18 October is World Menopause Awareness Day
 - October is Breast Cancer Awareness Month

- ☐ Training line managers to be aware of gender-specific health conditions and what adjustments may be necessary to support women at work.
- ☐ Undertaking equality impact assessments of all workplace policies and practices to ensure they take into consideration women's reproductive health and potential barriers affecting workers.
- ☐ Ensuring that all pregnancy and childbirth-related sickness absence is recorded separately from any other sickness absence by the employer so that the employee is not disadvantaged and it is not used as a reason for disciplinary or capability action, dismissal or redundancy.
- ☐ Flexible sickness absence procedures that do not penalise women staff for time off for other gender-specific health conditions such as menstrual and peri/menopausal symptoms.
- ☐ Return to work interviews that consider that a range of symptoms could be related to gender-specific conditions, with training for managers to deal with them in a supportive and sympathetic way.
- ☐ Time off (ideally paid) for medical screening (such as for cervical cancer), on-going treatment (such as for infertility) and check-up appointments to manage conditions (such as for HRT treatment) that are not recorded as sick leave.
- ☐ Encouragement of staff to have regular check-ups for HRT treatment, family planning issues, cervical and mammogram cancer screening etc.
- ☐ Provision of guidance on gender-specific illnesses and conditions in the workplace, and in wider occupational health awareness campaigns so that everyone knows that the employer has a positive attitude to the issues.
- ☐ An option for women and affected trans, non-binary or intersex employees to speak confidentially about their condition to a knowledgeable and confidential workplace point of contact such as someone from human resources or from an employee assistance programme, particularly if their line manager is male.
- ☐ The provision of flexible working hours or practices, and the allowance of temporary changes in work patterns such as splitting lunch breaks into multiple shorter intervals in order to deal with their symptoms and screening required.
- ☐ Inclusion of gender-specific reproductive health and wellbeing health issues in workplace risk assessments as well as a physical and mental health assessment.
- ☐ Making adaptations and adjustments to the workplace physical environment such as providing quiet spaces for rest and management of symptoms, easy access to toilet, changing and shower facilities, easy and free access to menstrual products, hygienic disposal available in all toilet and changing facilities, windows that open or provision of fans, comfortable uniforms made in breathable natural fabrics, the provision of blinds or curtains to block out bright sunlight, installing and maintaining appropriate lighting, easy access to cool drinking water.

- ❑ Involvement of men, women and non-binary staff in relevant decision-making bodies and in monitoring and reviewing of related processes. Ensuring that they consider that trans men, non-binary and intersex people can menstruate and experience menopause.

Contact your **regional education teams and / or LAOS** to find out what training and resources are available to assist you with negotiating with your employer or promoting the issues in this guide with your members <https://learning.unison.org.uk>

Further guidance is available from **bargaining support** www.unison.org.uk/bargaining-guides . If you have any feedback that could improve the content of the guide, contact bargaining support at bsq@unison.co.uk